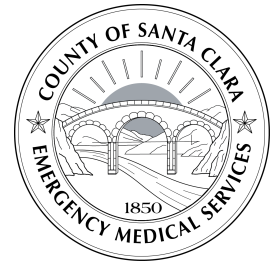


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Emergency Medical Services System

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Date: September 19, 2025

To: EMS System Stakeholders

From: Dustin Gonzalez, Program Manager
Prehospital Quality Improvement Unit 

Copy: Nicholas Clay, EMS Agency Director
Nicole D'Arcy, MD EMS Medical Director
Michael Cabano, EMS Agency Assistant Director

Subject: Santa Clara County EMS Protocol/Policy Summary of Changes

The EMS Agency has finalized the protocol and policy changes taking effect January 1, 2026. Education will be provided through the EMS Update online and in-person curriculum for Santa Clara County EMTs and paramedics. If you should have any questions or concerns regarding this memorandum, please contact dustin.gonzalez@ems.sccgov.org

Medication Changes	Policy Name	Description
Expanding Indications for Magnesium Administration	700-A07 Cardiac Arrest	Torsades (polymorphic ventricular tachycardia)
	700-A11 Respiratory Distress	In COPD/asthma if no change with albuterol and ipratropium, magnesium can be administered
	700-A14 Tachycardia with Pulses	Atrial fibrillation with Rapid Ventricular Response greater than 150 bpm Torsades (polymorphic ventricular tachycardia)
	700-A18 Gynecological and Obstetrical Emergencies	Added two new indications: (1) pre-eclampsia with severe features and (2) severe hypertension in pregnancy

	Policy 302 Minimum Inventory	Magnesium minimum inventory 4 gm
Tranexamic Acid	700-A16 Trauma Care	Increased dose to 2 gm
	700-A18 Gynecological and Obstetrical Emergencies	Post-partum hemorrhage 1 gm
	Policy 302 Minimum Inventory	TXA minimum increased to 2 gm
Push-dose Epinephrine	700-A10 Shock 700-P10 Shock	Push-dose epinephrine replacing dopamine to maintain blood pressure goal Clarified normal saline administration doses
	700-A04 Sepsis	Directed to follow 700-A10 Shock protocol for push-dose epinephrine
	700-A05 Bradycardia 700-P05 Bradycardia	Adults: Both TCP and push-dose epinephrine can be used as first line treatment for symptomatic bradycardia Pediatrics: Push-dose epinephrine replacing cardiac arrest dose epinephrine in bradycardia unresponsive to oxygen/ventilation
	700-A07 Cardiac Arrest 700-P07 Cardiac Arrest	Replaced dopamine for ROSC patients per 700-A10
Ipratropium (Atrovent)	700-A11 Respiratory Distress	In COPD/Asthma combine ipratropium with first dose of albuterol (single dose)

		SpO2 goal 88-92% for COPD
	Policy 302	Minimum inventory of ipratropium 0.5 mg
Midazolam	700-A02 Seizure 700-P02 Seizure	Adults: Specified 5 minutes between midazolam administrations Pediatrics: IM Midazolam dose doubled to 0.2 mg/kg Specified 5 minutes between midazolam administrations Base contact if more than 2 doses needed
	700-A20 Behavioral Emergency	Specified 10 minutes between doses of midazolam Removed Base Contact for second dose of behavioral sedation
Adult Treatment Protocol Changes		
Protocol	Change	Rationale for Change
Protocol 700-A05 Bradycardia	Increased albuterol maximum dose to 20 mg for suspected hyperkalemia	Align with standards of care
Protocol 700-A07 Cardiac Arrest	Traumatic cardiac arrest do not resuscitate if PEA less than 40 bpm/asystole or transport time greater than 20 minutes	State Trauma Advisory Committee and local trauma center recommendations
Protocol 700-A13 Stroke	Prioritized obtaining blood glucose measurement Added stroke activation window less than 24 hours	Align with standard of care and stroke center feedback

Protocol 700-A14 Tachycardia with Pulses	Specified threshold of 100 mmHg for sedation prior to synchronized cardioversion	Align with standards of care
Protocol 700-A16 Trauma Care	Moved TXA administration from 700-M17 Removed base contact for administration of TXA with continued hemorrhage despite tourniquet	Organization of protocols
Policies/Procedures/Standard Protocols		
Protocol/Policy	Change	Rationale for Change
Policy 410 Pediatric Receiving Center Standards	Revised from Pediatric Receiving Center Designation	Align with other specialty center standards policies and state regulation changes
Procedure 700-M02 Pleural Decompression	Clarified indications to perform pleural decompression for tension physiology (hypotension and peri-arrest)	Feedback from trauma care system stakeholders
Procedure 700-M17 Traumatic Hemorrhage Control	Moved TXA administration to 700-A16 Trauma Care	Clarity of treatment protocol
Protocol 700-S02 ALS to BLS Transition of Care	Removed references to SDOs Added "Acute onset of severe headache" as exclusionary Changed vital sign parameters to "At time of transport" Added IV acetaminophen to allowed medications to have been administered prior to transfer of care to BLS personnel Allow for BLS personnel to monitor and maintain IV normal saline.	Expand patients safely managed by BLS providers

Protocol 700-S04 Routine Medical Care - Adult	ETCO2 must be monitored after any administration of midazolam Pain scale score must be documented pre/post analgesic Consider ETCO2 monitoring after morphine administration.	Patient safety Address systemwide documentation deficiency
Protocol 700-S05 Routine Medical Care – Pediatric	ETCO2 monitoring must be attempted after any administration of midazolam Pain scale score must be documented pre/post analgesic	Patient safety Address systemwide documentation deficiency
Protocol 700-S06 Falls	If time of fall was more than 72 hours and no other Red Trauma Triage Criteria are met, do not activate as Trauma Alert Patient	Feedback from trauma care system stakeholders
700-S15 Abuse/ Assault/ Domestic Violence/ Human Trafficking	New protocol	The purpose of this protocol is to give guidance and guidelines for treating and transporting patients who are victims of elder or child abuse, victims of sexual assault, domestic violence or human trafficking.
700-S16 911 EMS Nurse Navigation: Field Personnel Referral	Moved from optional protocols with conclusion of pilot project	
700-S17 911EMS Nurse Navigation: Emergency Medical Dispatch Center Referral	Moved from optional protocols with conclusion of pilot project	
700-S17 Schedule A 911 EMS Nurse Navigation: Emergency Medical Dispatch Center Referral	Moved from optional protocols with conclusion of pilot project	

Summary of Medication Changes

	Indication	Dose	Timing	Drip rate
Magnesium	Eclampsia	4g in 100 ml	20 minutes	50 gtt/min (0.8 gtt/sec)
	Pre-Eclampsia with Severe Features			
	Severe HTN in pregnancy			
	A fib with RVR	2g in 100 ml	15 minutes	60 gtt/min (1 gtt/sec)
	Torsades (with pulses)			
	COPD/asthma			
	Torsades (pulseless)	2g in 10 ml	2 minutes	

	Indication	Dose	Timing	
TXA	Uncontrolled Extremity Hemorrhage	2 g in 100 ml	10 minutes	100 gtt/min (1.6 gtt/sec)
	Non-compressible Truncal Hemorrhage			
	Post-partum Hemorrhage	1 g in 100 ml		

	Indication	Dose	Notes
Ipratropium	Respiratory Distress (COPD/asthma) Adult	Ipratropium 0.5 mg, 1 dose	with Albuterol

	Indication	Dose	Timing	Max total
Midazolam	Behavioral Emergency Sedation	IM: 5 mg	q 10 minute	2 doses
	Seizure (Adult)	IM: 10 mg, 2nd dose 5mg	q 5 minute	15 mg
		IV: 2.5 mg IV		15 mg
	Seizure (Peds)	IM: 0.2 mg/kg		2 doses
		IV: 0.1 mg/kg		2 doses

	Protocols affected	Protocol change
Push-Dose Epi	Shock (Adult)	Replace dopamine
	Bradycardia (Adult)	Replace dopamine
	Sepsis (Adult)	Refer to Shock protocol if hypotensive
	Cardiac Arrest (Adult)	Replace dopamine in post-ROSC hypotension
	Shock (Peds)	Replace dopamine
	Bradycardia (Peds)	Replace cardiac dose epi with push dose epi (before atropine)
	Cardiac Arrest (Peds)	Replace dopamine in post-ROSC hypotension